



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : فاتن فرحات سعد غالى
المحافظة : البحيرة
الحى / المدينة : الدلنجات
العنوان : 0
رقم البطاقة : 27611041801906 رقم التليفون : 01147627482
العمر (تاريخ الميلاد) : 04-11-1976 المهنة : حاصل علي دبلوم
تاريخ تسجيل الإستمارة : 09-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فانت فرحات سعد غال National ID: 27611041801906	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient's Name: FATEN FARAHAT SAAD GHALY
Date: 2026-04-26
Referred by: Prof.Dr. CT CAP IV
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported left breast carcinoma , compared to last study dated on 28/9/2025.

(I) Chest:

- ☐ left mastectomy with clear operative bed , no masses or collection .
- ☐ Middle lobe subpleural reticulations and atelectatic bands are still noted likely post radiotherapy
- ☐ Few enlarged mediastinal lymph nodes , the largest seen at right paratracheal group measures 8 x 3mm .
- ☐ Regression of perifissural nodules previously noted pulmonary nodules .
- ☐ Clear both lung fields .no sizable pulmonary masses or consolidation patches
- ☐ No pleural or pericardial collections seen.
- ☐ No gross cardiac abnormality.

(II) Abdomen and pelvis:

- ☐ The liver is enlarged in size showing homogenous CT density , no focal lesions

Patient's Name: FATEN FARAHAT SAAD GHALY
Date: 2026-04-26
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- ☐ Patent and homogenously enhancing portal vein.
- ☐ No intra or extra hepatic biliary radicles dilatation.
- ☐ Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- ☐ Normal CT appearance of the spleen .
- ☐ Regular filling of the urinary bladder showing no stones, masses .
- ☐ Unremarkable other pelvic organs.
- ☐ No retro-crural, para-aortic or pelvic lymphadenopathy.
- ☐ No ascites.

No appreciable changes reagarding :

- ☐ Spondylodegenerative changes .
- ☐ Left iliac small sclerotic lesion likely bone island .
- ☐ No worrisome lytic or sclerotic bony lesions ..
- ☐ No vertebral structural collapse .

OPINION:.

- Regression of tiny pulmonary nodules .
- Rather stationary rest of the study

Dr.Reham Khedr, MD

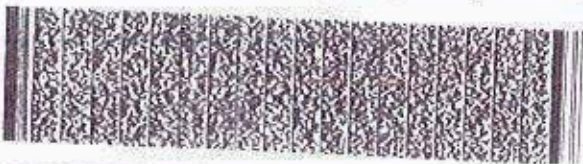
Name: فاتن فرحات سعد غالي			
MR. : 5130			
Detailed Medical Report			
		السيد	
تجبة طبية وبعد,,			
إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	
ننشر بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 60-kg, 157-cm individual with a history of left breast cancer, stage cT4b, HER2-positive (3+), initially treated with left modified radical mastectomy on 10/11/2021. Subsequent disease progression included brain metastases diagnosed in 2022, for which whole-brain irradiation was administered in September 2025 followed by stereotactic body radiotherapy in March 2025. The patient received adjuvant trastuzumab (Herceptin) and later transitioned to a regimen of trastuzumab IV 440 mg combined with capecitabine (Xeloda) 3 × 2 × 14. The most recent imaging in March 2025 demonstrated mild progression of a left parietal brain lesion measuring 2.5 × 1.7 × 1.7 cm. Follow-up in February 2026 confirmed ongoing metastatic breast cancer to the brain while the patient continued trastuzumab IV 440 mg and capecitabine 3 × 2 × 14. Laboratory studies were within normal limits, and an echocardiogram performed for cardiac monitoring showed an ejection fraction of 60 %. A brain MRI was pending at that time. In March 2026, the same regimen was continued with trastuzumab IV 440 mg and capecitabine 3 × 2 × 14; the echocardiogram remained stable at EF 60 %, and a brain MRI remained pending. An MRI of the brain performed in early April 2026 demonstrated mild progression of the known left parietal lesion. The patient had previously received whole-brain irradiation in September 2025 and SBRT in March 2025. Multidisciplinary discussion recommended a change in systemic therapy to a combination of lapatinib (Tykerb) and vinorelbine (Navelbine), pending approval. On 22 April 2026, the multidisciplinary team reviewed the case and planned initiation of lapatinib/vinorelbine; approval for lapatinib was still pending. A CT of the chest, abdomen, and pelvis was performed that day. By 10 May 2026, lapatinib approval remained pending. The patient was started on vinorelbine 100 mg on days 1 and 8, and a CT CAP performed in April 2026 showed regression of pulmonary nodules, consistent with stable disease. On 8 June 2026, the patient was due for cycle 2 of the vinorelbine/lapatinib regimen. The patient reported improvement in confusion and convulsions. On 6 September 2026, the patient continued receiving vinorelbine 90 mg every three weeks (although the intended schedule is every four weeks) in conjunction with lapatinib. The patient has completed two cycles of the vinorelbine/lapatinib regimen to date.			
تحريرا في ,			
التاريخ :		19-09-2026	
الوقت :			
الطبيب المعالج :		هنا اسامه عبد الرحيم عبد البر	
يعتمد ,			



٢٠٢٢/١٠ ٢٧٦١١٠٤١٨٠١٩٠٦
حاصلة على دبلوم صناعي



أنثى مسلمة
ابراهيم مبروك خضر شحاته
البطاقة سارية حتى ٢٠٢٩/١٠/٢٤



جمهورية مصر العربية
بطاقة تخفيف الشخصية

فائق
فرحات سعد شالي
ش عرابي
مركز التلغات - البحيرة

٢٧٦١١٠٤١٨٠١٩٠٦

٢٧٦١١٠٤١٨٠١٩٠٦

J09451642

04/12/2026

MDT FORM

Name:	Faten Farahat Sayed Ghaly	MRN:	5130
Age:	49 Y	ID:	27611041801906
Menopausal Sta	PREMENOPAUSAL	Phone:	01147627482
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:	IDC (ps 2-3)
ER:	-VE
PR:	-VE
HER2:	+3
KI67:	

R

L

Sonomammography:

Mets Workup:

MRI BRAIN --> DP

History:

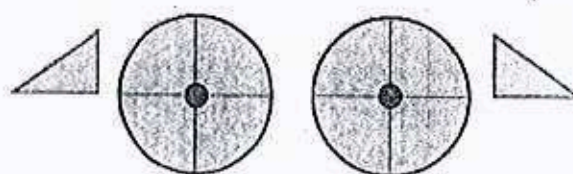
A CASE OF FEMALE PT WITH LABC PT RECIVED NACT TCHP X6 THEN UNDERWENT SURGERY THEN RECEIVED TDM1 --> 9-2024 --> BRAIN METS --> PT RECEIVED HERCEPTIN IV + XELODA PT CLINICAL DP PT WAS RECEIVED WBI AND SBRT --> FOR SHIFT TO TYKERB + CARBO +/- GEMZAR

Recommendations:

SHIFT TO TYKERB + CARBO +/- GEMZAR

MDT Outcome & Plan

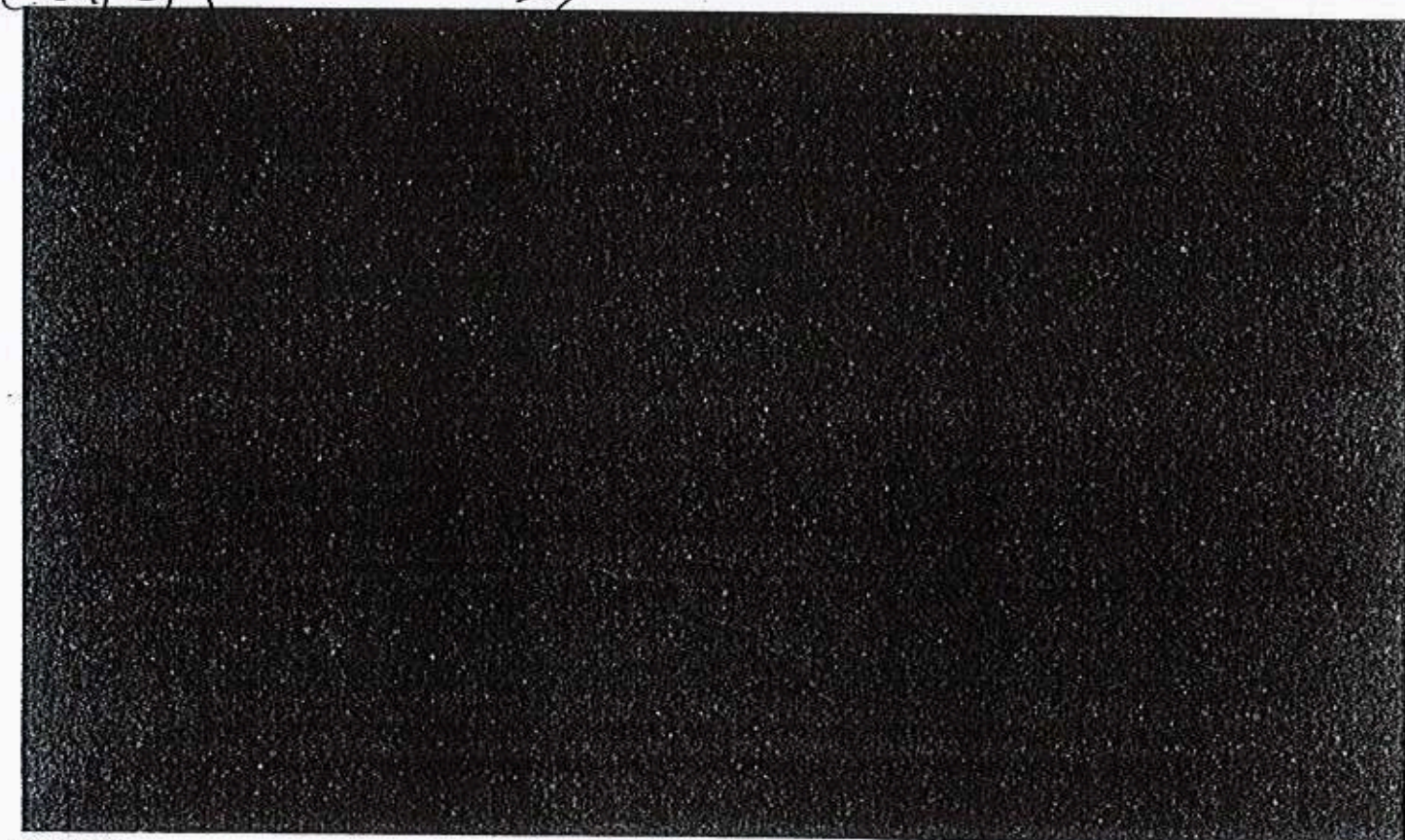
TYKERB + NAVELBINE



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. نورهان مصطفى					



about:blank

MDT FORM

٩ / ٩ / 2025 G

Name	فاندرات	MRN	1023/64
Age	45+	ID	CV211.61N.19.7
Menopausal status	premen	Phone	01147627482
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

MDT

Clinical Data

BX:	INDC	R	PS2-3	L
ER:	PR:	HER 2:	Ki67:	
-ve	-ve	3+		
Sonomammography:		premenopausal Herceptin Breast & Brain		
Mets Workup:		CTub.No, NM		
Histy: Left Breast		Ree NACT & CTup		
on 10/11/2021 → Left Mast 4 pto 4 pN1 Y10		Ree 12cyds Kaelce x12br 9/2022		
Recommendations:		9/224: Mets to Brain: 30cm. wct		
Ree Tard + Herceptin		7cy → grade 3pN st. Xeloda + Herceptin		
ET HNE Brain 12/2024, 3/2025: RD		4/2025: ↑ fets HNE Brain 1/7/2025: progren in		
MDT OUTCOME & PLAN		size of Efr pericth intracranial sol		
m 2.8x1.5x1.9		Op 4/2025: fets		
4/2025: 2.8x1.5x1.9		MRI 2/19/2025: Improment		
ESBT on sing braket		can fin Xeloda / Herceptin		

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. Kery					



Patient Name	: الأستاذة / فائق فرحات سعد غالى	Request No.	: 135641
Patient No.	: 27611041801906	Request Date	: 10/11/2021
Path No.	: 2716/21	Verification Date	: 14/11/2021
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referring Doc :			

Pathology Report

Clinical Data:

Left breast. Post neoadjuvant chemotherapy

Nature of the specimen:

Left MRM

Gross Examination:

Two specimens were received:

- 1- Left MRM specimen measured 17x13x4cm, covering skin 18x11cm, nipple was unremarkable grossly. Axillary fat measured 6x4cm. Cut section revealed an ill-defined firm greyish white tumour bed site measured 3x2x2cm, 2cm away from the skin and 1cm from deep margin. Axillary fat dissection revealed 9 lymph nodes up to 1cm.
- 2- Apical lymph node specimen: A fibrofatty tissue piece measured 1.5x1cm containing one lymph node 0.5x0.3cm

Microscopic examination:

- Histological type: No viable tumour tissue. Dilated ducts with necrotic material, foamy macrophages and focal calcification. Chronic inflammatory reaction and fibrosis.
- Grade: N/A
- Angioinvasion: N/A
- Ductal carcinoma in situ: N/A
- Lymph node involvement: Positive metastatic deposits in (1/10) lymph nodes, with perinodal fat infiltration.
- Resection Margins: Free deep margin
- Histologic response to therapy: Pathological complete response.
- Additional pathologic findings: Free nipple and areola

Diagnosis:

Left MRM:

No viable tumour tissue.

Pathological complete response.



Pathology Report

Name: فائق فرحات سعد غالي

Age: 44years.

Clinical data: Left breast mass(as per the involved report)..

Specimen type: Slid, revision.

Date (Received):19/4/2021

Date(reported): 28/4/2021

Lab. Number :SR-158/21

Hospital Number: /21

Gross Description:

Four H&E stained slides labelled as (2452/21) , four immunostained slides (ER- PR- HER2- KI67).

Microscopic Examination:

histopathological examination of the slides received reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II].

- Examination of ER stained slide show No nuclear staining of the tumor cells.
- Examination of PR stained slide show No nuclear staining of the tumor cells.
- Examination of HER2 stained slide show intense membranous staining in > 10% of tumor cells.
- Examination of the KI67 stained slide show proliferation in 60% of the tumour cells.

Diagnosis:

Left breast mass, Slide revision;

- Invasive duct carcinoma grade III.
- ER: Negative (0/8).
- PR: Negative(0/8)
- Her2: Positive . Sco. (3+).
- KI67: proliferation in 60% of the tumor cells.

Prof. Dr. Amal Hareedy.

Dr. Marwa Mahmoud.

Dr. Mona Mohamed.

FATEN FARAHAT SAAD GHALY

Name:
Date of Examination: 10-9-2024
Referring Physician: Prof.Dr.

Examination:
MRI of the brain with contrast

Findings:

Evidence of a ring enhancing partially cystic lesion at the left high parietal convexity region showing no restricted diffusion measuring 3x2.5x2.5cm with surrounding grade II perifocal edema yet no significant mass effect... in view of the clinical data, the current lesion is considered metastatic lesion... for clinical correlation and further work-up.

- No other similar lesions.
- Normal size and shape of the supra and infra tentorial components of the ventricular system.
- No evidence of abnormal intra axial fluid collections.
- No midline shift
- Normal morphological features of the cerebellar hemispheres with no focal mass lesions or areas of abnormal signal changes. Normal appearance of the CP angles.
- Normal components of the brain stem.
- Normal sellar, parasellar and suprasellar regions.
- Scans through the calvarium are unremarkable.

Dr. Ahmed El-Sabaa, MD



Patient Name: Faten Farahat Saad Ghaly	Date : 16/09/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 48 Year(s)
Medical ID: 1533/2021	Pat. ID: 24783

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

****A Reported Case of Left MRM with no Available Previous Study**

Findings:

I Chest: -

- Left MRM with clear operative bed with no collections, masses or pathologically enlarged ipsilateral axillary lymph nodes.
- Left lobe shows subpleural reticular bands noted at the superior lingula likely post-radiation pneumonitis.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Few prominent right axillary lymph nodes with largest measuring 14x9mm, showing preserved fatty hilum, likely reactionary.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, thickening or masses seen.
- Borderline cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is mildly enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenal glands.



Patient Name: Faten Farahat Saad Ghaly	Date : 16/09/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 48 Year(s)
Medical ID: 1533/2021	Pat. ID: 24783

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

III-The scanned skeleton:

- Spondylodegenerative changes with multilevel schmorls nodes and subchondrial sclerosis.
- The scanned skeleton shows left Iliac bone sclerotic focus measuring 5 mm likely bone island, yet no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

Opinion:

- Left MRM with clear operative bed with no collections or masses.
- Left lobe subpleural reticular bands likely post radiation pneumnitis.
- No CT evidence of pulmonary, hepatic or osseous metastatic deposits.

Dr. Aya Ahmed

Dr. Sherif Sameh, M.D.



Patient Name: Faten Farahat Saad Ghaly	Date : 07/04/2025
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 48 Year(s)
Medical ID: 1533/2021	Pat. ID: 24783

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenal glands.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

III-The scanned skeleton:

- Spondylodegenerative changes with multilevel schmorls nodes and subchondrial sclerosis.
- Still noted: left iliac bone sclerotic focus measuring 5 mm likely bone island, yet no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

Opinion:

- Left MRM case show : No interval time changes in current study.
- Stationary course since last study with no CT evidence of pulmonary, hepatic or osseous metastatic deposits.

Dr.Maitha Fadhl

Much obliged,
Dr. Reham Anwar, FEBR, MD

Patient Name English: Faten Farahat Saeed Ghaly	Age: 48 Year(s), 7 Month(s)
Study Date: 01/07/2025	Sex: Female
Ref. Phys.: *Dear Colleague*	Procedure: MRI Brain With Contrast

Patient with history of metastatic breast cancer , in view of the report and hard copy of the pervious study dated 3-2025, the current study revealed:

- *Progression in size of the previously noted left high parietal intra axial lesion currently averaging 2.8x1.5x1.9cm showing heterogenous mainly peripheral enhancement and restricted diffusion.*
- *Progression of the previously noted moderate perifocal edema with mild mass effect upon the related cortical sulci.*

Otherwise

- Normal morphological features of the cerebral and cerebellar hemispheres.
- Normal supra and infra tentorial ventricles as well as the sub-arachnoid spaces.
- No abnormal intra- or extra- axial fluid collections.
- No midline shift.
- Normal cerebello-pontine angles and brain stem.
- Normal bony calvarium.

Dr. Maha Kotb, MD.
Consultant radiology, Alexandria university.



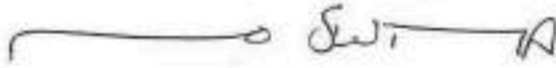
Echo report

Name: فانتة فرحات
Date: 15/10/2024

MR: 1021
C.92

EF	67.1	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.7	3.5-5.8cm	AO.R	2.8	2-3.7cm
LVESD		2-4cm	LA	3.1	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

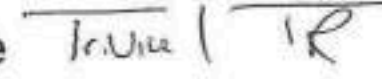
LV Dimensions 
LV wall thickness 
LV diastolic function DD grade II

SWMA 

LA and Aortic root dimensions 

Mitral Valve 

Aortic valve 

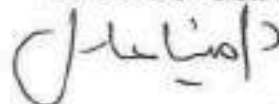
Tricuspid valve 

Pulmonary valve 

Right side and systolic function 

Intracardiac masses and effusion 

D/Msc cardiology



Medical No. : 10483

Patient's Name: Faten Farhat Saad Chali

Date: 2026-09-27

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



0/2.

2021
20

Examination:

MRI of the brain with contrast

Technical data:

- Routine pulse sequences
- IV contrast was given

Indication: known case of breast cancer with brain mets

Comparison: 30-11-2025 report

Findings:

- Progressive size of the left high parietal para sagittal mass lesion 4.5X3X2.6 cm compared to 2.5X1.7X1.7 cm at the last report. With remarkable surrounding edema (grade III) + related 5 mm midline shift to the right.

The lesion exhibits heterogeneous signal with few small hemorrhagic foci and heterogeneous mainly peripheral enhancement.

- Brain involutinal changes, in the term of mild symmetric ventricular dilatation and prominent extra-axial CSF spaces.
- Bilateral cerebral patchy areas of altered MRI signals (low T1, high T2 and FLAIR signals), involving the deep peri-ventricular white matter and centrum semiovale
- Bilateral cerebral white matter multiple small foci of abnormal signal (high T2 and FLAIR signals) with no perifocal edema or mass effect
- Normal MRI appearance of the cerebellum and brain stem with no areas of MRI signal abnormalities.

Conclusion:

- Progressive size of the left high parietal para sagittal mass lesion 4.5X3X2.6 cm compared to 2.5X1.7X1.7 cm at the last report. With remarkable surrounding edema (grade III) + related 5 mm midline shift to the right.
- Age related cerebral involutinal changes.

Medical No. : 10483

Patient's Name: Faten Farhat Saad Ghali

Date: 2026-09-27

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



- Bilateral cerebral ischemic leukoencephalopathy with multiple bilateral old lacunar infarcts (Fazekas grade I-II)

Regards, Dr. Ahmed Rezk, MD

Ahmed

Patient's Name:
Date:
Referred by:
Hospital:

FatenFarahat
2026-09-06
Prof.Dr. no doctor
Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

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- Middle lobe subpleural reticulations and atelectatic bands are still noted likely post radiotherapy.
- Few enlarged mediastinal lymph nodes, the largest seen at right paratracheal group measures 8 x 3mm.
- Regression of peritissural nodules previously noted pulmonary nodules.
- Clear both lung fields, no sizable pulmonary masses or consolidation patches.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density, no focal lesions.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- Normal CT appearance of the spleen.
- Regular filling of the urinary bladder showing no stones, masses.
- Unremarkable other pelvic organs.
- No retro-cruural, para-aortic or pelvic lymphadenopathy.
- No ascites.

No appreciable changes regarding:

- Spondyloddegenerative changes.
- Left iliac small sclerotic lesion likely bone island.
- No worrisome lytic or sclerotic bony lesions.
- No vertebral structural collapse.

Patient's Name:

Faten Farahat

Date:

2026-09-06

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

OPINION:

- Regression of tiny pulmonary nodules.
- Rather stationary rest of the study.
- Current study: CT abdomen and pelvis with IV contrast is showing stationary course.

Best regards,

Dr. Abdulaziz Oraby, MD.

Lecturer of interventional and radio-diagnosis.